



GENEVIEVE App™

GENEVIEVE HEALTH™ | Mental Health Practice Support



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**Safety from roots to every
journey**

A LETTER TO IRENE

What GENEVIEVE HEALTH™ can place in your hands
tomorrow - and what it can become for your practice

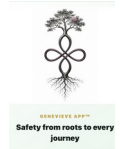
Prepared by Tracey Ann Kennedy
17 July 2026





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Dear Irene,

You have just returned from a well-earned month away and discovered that a staff member is unavailable for two weeks, while the full picture of what has happened with staff, clients, appointments and follow-up work is still not sitting in one clear place. That is exactly the kind of gap I want GENEVIEVE App™ to help close.

I am not offering you a complicated clinical replacement system overnight. I am offering you a practical, working operational program that can be deployed tomorrow as a controlled practice-support pilot. It will help your staff see what needs attention, help reception know what to do, help management understand what has changed and help protect clients from being forgotten between appointments, messages, leave and emergencies.

The first release will be deliberately kept within a safer legal and professional boundary. It will support people and approved procedures; it will not diagnose anyone, decide whether a person is safe, replace a clinician, provide treatment, or independently contact emergency services. Real clinical records should only be entered after you approve the privacy, access, security, clinical-governance and emergency procedures. The initial pilot can use fictional or de-identified information, or limited non-clinical operational information that you approve.

What I can give you as a working program tomorrow

Irene's live command centre: A clear opening screen showing who is working, who is away, what changed while you were absent, today's appointments, overdue callbacks, unallocated work, serious alerts and decisions requiring you.

Staff absence and coverage: Planned leave, sudden sickness, flooded roads, transport problems, late arrivals and emergency absences can immediately identify affected appointments and tasks, nominate coverage and require the covering person to accept responsibility.

Return-from-leave handover: A structured briefing of staff changes, missed appointments, unresolved messages, reports, referrals, incidents, client transfers and tasks that remain open.

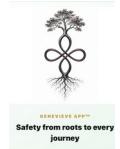
Reception support: A simple queue for calls, voicemails, appointment changes, referrals and distressed callers, with safe scripts and one-button escalation to an authorised practitioner. Reception will not be expected to make clinical decisions.

Client continuity register: A visible list of clients who missed an appointment, cancelled without rebooking, have no next appointment, are waiting for allocation, have an unanswered message or are affected by a clinician's absence.



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Green, amber and red escalation: Every important matter receives an owner, deadline and backup. Green means on track, amber means attention is approaching, and red means urgent, overdue or unaccepted.

Storm, flood and closure controls: One place to record severe weather, inaccessible roads, power or internet failure, building closure and staff availability, then identify affected clients and prepare approved notifications.

Incident and urgent-safety control: A clearly visible Immediate Safety Concern button that assigns the issue to the authorised clinician and opens Irene's approved procedure. It will record who accepted the matter, what was done and the outcome.

Daily opening and closing checks: Reception and management can confirm staffing, phones, inboxes, appointments, unresolved red alerts, building safety and end-of-day handovers.

Audit and accountability: The program records who saw, accepted, changed and completed each important task so matters cannot quietly disappear.

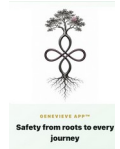
The central GENEVIEVE rule:

**DETECTED → ASSIGNED → ACCEPTED → ACTIONED → OUTCOME RECORDED → REVIEWED
→ CLOSED**



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How tomorrow's program will take care of everyone

Irene and management

A truthful, current picture without having to search calendars, emails, messages and people's memories. The system highlights what needs Irene personally and what has already been safely delegated.

Clinicians

Clear handovers, accepted coverage, organised follow-ups, visible deadlines, safer return from leave and protection from tasks being silently transferred to them.

Reception

Simple operational instructions, safer call handling, clear escalation and enough information to help without exposing unnecessary therapy notes.

Clients and patients

Fewer missed follow-ups, faster notice of appointment disruption, clear ownership of callbacks and stronger continuity when a staff member is absent.

Staff wellbeing

Workload visibility, breaks, supervision reminders, debrief requests, working-alone check-ins and support after serious incidents.

The practice

A time-stamped record of decisions, handovers, actions, incidents, closures and unresolved risks.

What will not be switched on tomorrow

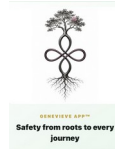
To protect the practice, staff and clients, the first release will not:

- store unrestricted therapy notes or full clinical files before security and access controls are approved;
- diagnose, score or predict suicide risk;
- declare a client safe because they replied, or unsafe merely because they missed an appointment;
- send sensitive messages without approved wording and communication permissions;
- automatically contact police, family or emergency contacts;



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- use artificial intelligence on identifiable clinical information without appropriate approval, consent and privacy arrangements;
- replace your existing practice-management, clinical-record or emergency systems without a planned integration and migration process.

What GENEVIEVE App™ can bring once the practice is back on its feet

A complete practice operating picture: Staffing, appointments, referrals, waitlists, documents, incidents, workloads and business continuity connected in one controlled view.

A secure client support portal: Appointments, approved forms, consent, communication preferences, callback requests, accessible information and the client's approved safety-plan view.

Integrated referral and waitlist management: No referral lost in an inbox; every referral reviewed, allocated, redirected or closed with a recorded outcome.

Telehealth preparation and backup: Identity, current location, backup phone contact, privacy, consent and connection-failure procedures prepared before the session.

Clinical-document workflow: Tracking for notes, reports, letters, treatment plans, signatures, consent and delivery confirmation - while keeping final clinical responsibility with the practitioner.

Critical-incident centre: Structured support for suicide-related incidents, serious self-harm, violence, privacy breaches, emergency-service attendance, deaths, staff injury and post-incident review.

Staff governance and wellbeing: Supervision, credentials, insurance, training, leave, fatigue, workload, debriefing and policy acknowledgement.

Business intelligence: Patterns showing where delays repeatedly occur, where staff need support, where reception is overloaded and where systems should be improved - without turning people into numbers.

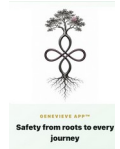
Controlled integrations: Connection with approved calendars, secure messaging, practice-management systems, telehealth, accounting and reporting tools, subject to technical and legal review.

Growth without losing the human heart: A system that can expand with the business while keeping Irene, staff, reception and clients visible, supported and accountable.



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What I need from you

To make tomorrow's pilot useful and safe, I need a short working session with you to confirm:

- the staff roles and who is allowed to see each type of information;
- your current leave, handover, non-attendance and emergency procedures;
- the clinical lead and backup escalation people;
- approved wording for appointment disruption and callback messages;
- which information may be used in the first pilot;
- the existing program or ZIP that you want preserved as the foundation;
- the devices and computers staff will use;
- the one or two workflows causing the most immediate harm or confusion.

I want this to reduce pressure rather than create another system for you to feed. The design will therefore begin with the information staff already use, make the urgent work visible and keep every action simple enough to perform during a busy or stressful day.

**My promise is not that technology can prevent every tragedy or mistake.
My promise is that GENEVIEVE App™ will be built so warning signs, responsibilities and unfinished actions are far harder to lose.**

This program is about taking care of everyone: the person asking for help, the receptionist who answers the call, the practitioner carrying the clinical responsibility, the staff member who needs leave, and the manager who must understand the whole picture.

I would be honoured to place the first working version in your hands tomorrow and then build it carefully with you into the full GENEVIEVE HEALTH™ practice-support system.

Yours sincerely,

Tracey Ann Kennedy

Founder and Inventor

GENEVIEVE App™

Implementation note: This is a controlled operational pilot, not legal or clinical advice. Identifiable client information must not be used until Irene approves the practice's privacy, security, access and clinical-governance arrangements.